National Cancer Control Programme (NCCP) 00858 · clinician draft

Pembrolizumab 200 mg + weekly carboplatin AUC 1.5 / paclitaxel → AC

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Phase 1 · weekly carboplatin / paclitaxel

Active medicines: Carboplatin + Paclitaxel + Pembrolizumab on its separate schedule

Emetogenic risk: Moderate · carboplatin AUC <4

Medicine	Dose / route	When
Ondansetron	8 mg orally twice daily	Day 1 National Cancer Control Programme (NCCP) alternative to 16 mg once daily; review QT / hepatic risks
Dexamethasone	8 mg orally	Day 1; reconcile paclitaxel premedication Do not add to the paclitaxel steroid course automatically
Dexamethasone	8 mg orally daily	Days 2–3 reference antiemetic row Reconcile the delayed period with paclitaxel steroids; one final individualised steroid prescription
Metoclopramide	10 mg orally up to three times daily as required	Subsequent days: duration to be confirmed Review contraindications and renal function

Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before treatment.
- Day 8 and later: no routine dexamethasone unless the consultant specifies it for reaction/risk; chlorphenamine continues and any famotidine omission requires consultant/local-policy agreement.
- Reconcile antiemetic and taxane dexamethasone into one prescription; never add duplicate steroid courses.
- Four 21-day cycles: paclitaxel on Days 1, 8 and 15; carboplatin AUC 1.5 on Days 1, 8 and 15. Pembrolizumab 200 mg on Day 1 of each cycle (every three weeks).
- This is a weekly combination visit, not paclitaxel alone.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00858 v3 · p9 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-20 · SACTCheck v0.84.13 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Phase 2 · AC

Active medicines: Doxorubicin + Cyclophosphamide + Pembrolizumab on its separate schedule

Emetogenic risk: High · AC

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1
Ondansetron	16 mg orally	Day 1 Individual dose / QT / hepatic review
Dexamethasone	12 mg orally	Day 1 No routine Days 2–4 dexamethasone in this National Cancer Control Programme (NCCP) AC antiemetic row
Olanzapine	5 mg orally	Days 1–4 Review sedation, falls, interactions and glucose
Aprepitant	80 mg orally daily	Days 2–3
Metoclopramide	10 mg orally up to three times daily	Subsequent days: duration and use to be confirmed Caution with olanzapine: additive extrapyramidal effects; adjust / avoid for individual contraindications

Additional phase-specific support

- Four further cycles (cycles 5–8): doxorubicin and cyclophosphamide on Day 1 every 21 days. Pembrolizumab 200 mg on Day 1 of each AC cycle.
- Assess granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) using this source and individual risk; do not silently copy a dose-dense schedule.
- Confirm cumulative doxorubicin exposure, cardiac assessment and venous-access plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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After surgery · pembrolizumab alone

Active medicines: Pembrolizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- After surgery: pembrolizumab 200 mg every 21 days for nine doses OR 400 mg every 42 days for five doses. These are alternative schedules, not consecutive courses. Progression preventing definitive surgery or unacceptable pembrolizumab toxicity changes this plan; postoperative treatment is not automatic.
- No routine cytotoxic antiemetics, taxane premedicines or chemotherapy granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) are carried forward.
- Continue the immune-toxicity and required blood/hormone monitoring.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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